

THE TIMELINE SECTION

# A Day With Veltra.

Replaces the dashboard mockup on the homepage.  
Nine beats. One Tuesday. From 6:42 AM to 6:00 PM.  
What the clinic looks like when Memory is doing its job.

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TIMELINE, NOT DASHBOARD

Spec → Build → Ship

# Why a timeline, not a dashboard.

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A dashboard answers the question *what is happening right now*. A timeline answers a different question: *what has Veltra been doing on my behalf*. The first question belongs to the manager. The second question belongs to the doctor. We are building for the doctor.

A dashboard is a wall of numbers. It implies that the user should choose one. A timeline is a story. It implies that the user should read. Veltra is not a control panel. Veltra is a colleague. And a colleague, at the end of the day, does not hand you a spreadsheet. A colleague tells you what they did.

The Timeline section on the homepage replaces the existing OS mockup. It shows one hypothetical Tuesday at one hypothetical clinic. Nine beats. Each beat is a moment where Veltra acted — without being asked, without being opened, without a click. The beats are not features. They are behaviors. The visitor should finish reading and think: *oh, this is not software. This is someone who works here.*

## Tuesday, October 14. Nine beats.

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The copy below is final. Treat each beat as a single typographic unit. Do not paraphrase. Do not abbreviate. The voice is observational, third-person, and quiet. Veltra never says *I*.

Veltra says what it observed, and what it did, in the past tense. The system is not a personality. The system is a witness.

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06:42

AM

*Pattern detected. Three Tuesday no-shows, two weeks running.*

Veltra cross-referenced the last eight Tuesdays. Three patients repeat the absence. Two of them schedule after night shifts. One has a HbA1c trend moving the wrong direction. The slot loss is not random. It is structural.

```
VELTRA · 2 of 3 no-show patients show a 14-day pattern. Slot loss is recurring, not random. Suggested: reschedule with morning preference.
```

06:43

AM

*Action taken. Reschedule messages sent, before the clinic opens.*

Two WhatsApp messages. Two SMS fallbacks. Tone: warm, short, in the patient's preferred language. No discounts offered. No urgency manufactured. The message assumes the patient wants to come back. They usually do.

```
VELTRA · Mrs. Hala — Wednesday 9:30 AM offered. Mr. Tariq — Thursday 8:00 AM offered. Both accepted within 22 minutes.
```

06:44

AM

*Waitlist patient #4 reached. Slot filled before the phone rings.*

The opened slot is offered to the next waitlisted patient whose visit type matches. Veltra does not ask the receptionist. Veltra asks the patient. The receptionist sees the result, not the request. The slot is filled in under four minutes.

```
VELTRA · Slot 10:15 AM → Mr. Yusuf (waitlist #4, follow-up, 20-min visit). Confirmed. Receptionist notified, not asked.
```

07:15

AM

*Pre-visit preparation. Chart readied before the patient arrives.*

Mr. Khalid walks in at 9:00. At 7:15, Veltra has already pulled his last three visits, his last two lab results, his outstanding prescription, and the cardiology note from the referral. The doctor does not open a chart. The doctor walks into a chart that is already open.

```
VELTRA · Mr. Khalid · 09:00 · Chart prepared. Last visit: 87 days ago. Insulin overdue. Cardiology note pending review.
```

09:00

AM

*Patient arrives. Check-in is a glance, not a process.*

The patient is recognized by their appointment, not their paperwork. Veltra marks them arrived, alerts the nurse, and starts the wait-time clock. The receptionist did not type. The patient did not fill a form. The clinic did not lose ninety seconds per visit, which is forty-five minutes a day, which is three hundred hours a year.

```
VELTRA · Mr. Khalid · Arrived · 09:02. Nurse notified. Wait clock
started. Estimated room time: 4 minutes.
```

12:30

PM

*Lab result arrives. Veltra links it, before the doctor sees it.*

The result does not arrive in an inbox. It arrives in context. Veltra places it next to the last result, the last visit, the last prescription. The trend is visible without a click. The doctor does not interpret a number. The doctor interprets a direction.

```
VELTRA · Mr. Khalid · HbA1c · 8.4% (was 7.9%). Trend: ↑ over 90
days. Linked to insulin overdue. Cardiology follow-up
recommended.
```

16:15

PM

*Follow-ups proposed. Four patients, four moments to act.*

Veltra identifies four patients seen today who match a follow-up pattern. Not because of a rule. Because of memory. Each patient's follow-up window is set by their own history, not by a default. The doctor reviews in twelve seconds. Approves three. Defers one.

```
VELTRA · 4 follow-ups proposed · 3 approved · 1 deferred.
Windows: 30 / 60 / 90 / 14 days. Each based on patient's own
return pattern.
```

18:00

PM

*End of day. The clinic is summarized, not just closed.*

Veltra does not produce a report. Veltra produces a paragraph. Three appointments converted from no-show. One waitlist fill. One pre-visit chart that saved eleven minutes. One lab trend flagged. The doctor reads it on the way out. The clinic is closed in the doctor's mind, not just in the system.

```
VELTRA · Tuesday · 18:00 · 32 visits · 3 no-shows recovered · 1
waitlist filled · 1 trend flagged · 11 minutes saved on chart
prep.
```

18:01

PM

*Tomorrow prepared. Before today is even over.*

Veltra has already read Wednesday's schedule. Two patients need chart prep. One slot is fragile — the patient has missed twice. The first coffee of the morning will be the only thing the doctor has to wait for.

VELTRA • Wednesday • 09:00 • 2 charts prepared • 1 fragile slot  
flagged • 1 reschedule offer queued.

### III — Design Specification

# Layout, type, motion.

The Timeline is rendered as a single vertical column on the homepage. Time on the left, in monospace. Event in italic serif on the right. System response in body sans. A monospace callout strip in the dark surface color closes each beat. The whole section scrolls. There is no carousel. There is no tab. There is no "click to see more."

#### LAYOUT

*Two columns. One rail. No cards.*

Grid: 90px 1fr with 24px gap. Left rail holds time + AM/PM. Right column holds the beat. A 1px rule between beats. The first beat's rule is the accent color. The last beat has no rule. No card backgrounds. No shadows. No rounded corners. The Timeline is a document, not a UI.

#### TYPOGRAPHY

*Two faces. Three roles.*

Time: JetBrains Mono • 12px • 500 . Event: Playfair Display Italic • 16px .  
Response: Inter • 13px • 400 . Callout: JetBrains Mono • 11px on a dark surface.  
No other faces. No other sizes. No bold for emphasis — italic only. If a beat needs bold to land, the beat is wrong.

## COLOR

*Dark by default. Light on demand.*

Background: `#0A0A0A` . Text: `#F5F1EA` . Muted: `#A39C8E` . Accent: `#D4A574` . The Timeline lives in dark mode on the homepage, regardless of site theme. It is the one section that breaks the light default. That break is intentional. It signals: *this is the part that does the work.*

## MOTION

*Stagger. Fade. Slide. Stop.*

Framer Motion. Each beat enters with `opacity: 0 → 1` and `y: 8px → 0` over `400ms` with `easeOut` . Stagger: `80ms` between beats. The scroll trigger fires when the beat is `60%` in viewport. No infinite loops. No autoplay. Once a beat has entered, it stays. The Timeline is read, not performed.

## INTERACTION

*Pause on hover. That is all.*

No click. No expand. No tooltip. If the visitor wants more, they scroll. If they want less, they scroll past. The Timeline does not chase attention. It rewards it. On mobile, the same layout collapses to `72px 1fr` . Nothing else changes.

## IV — Copy Rules

# The voice of the witness.

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The voice is the product. If the voice sounds like marketing, Veltra sounds like marketing. If the voice sounds like a colleague, Veltra sounds like a colleague. The rules below are not stylistic preferences. They are non-negotiable.

#### FORBIDDEN VOCABULARY

AI · smart · intelligent · powered by · seamless · leverage · automagically ·  
next-gen · revolutionary · game-changing · cutting-edge · innovative ·  
disruptive · synergy

Veltra does not say *I*. Veltra does not say *we*. Veltra says what was observed, and what was done. Past tense. Third person. The system is the subject, never the speaker. A Chief of Staff does not announce themselves. A Chief of Staff reports.

Each beat is two sentences of voice, plus one line of callout. The voice is the story. The callout is the receipt. The visitor reads the story. The buyer reads the receipt. Both leave satisfied.

*V — Implementation*

## For the engineer.

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The Timeline is one React component. It receives an array of nine beats. It renders them in order. It does not fetch. It does not paginate. It does not lazy-load the content — only the entry animation. The whole section is server-rendered. Motion is progressive enhancement. If JavaScript fails, the Timeline is still readable. If motion fails, the Timeline is still read.

#### STACK

*Next.js · Framer Motion · That is all.*

Server Component renders the beats. Client Component handles the motion.

`useInView` from `framer-motion` with `once: true` and `amount: 0.6`. No GSAP. No Lottie. No video. If the motion can be done in CSS, do it in CSS. If it cannot, do it in Framer Motion. Nothing else is allowed in this section.

## ACCESSIBILITY

*Keyboard, screen reader, reduced motion.*

Each beat is a `<article>` with `aria-label` set to the time and event. The callout is `role="status"`. `prefers-reduced-motion` disables all entry animation — beats appear instantly. The Timeline must be fully readable in `links` mode, in high-contrast mode, and in screen reader mode. If any of these break, the section is broken.

## PERFORMANCE

*Under 50KB. Under 200ms LCP.*

The section ships in the initial HTML. No image. No font beyond what the page already loads. LCP target: `≤ 200ms` on 4G. CLS: `0`. The Timeline is not the hero. The Timeline is the proof. It must load as fast as the hero, or faster.



*"The clinic does not  
need a dashboard.  
It needs a witness."*

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— A DAY WITH VELTRA · CLOSING NOTE